Cobimetinib + vemurafenib — advanced melanoma

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Advanced melanoma · NCCP 00373 v3 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. electrocardiogram (ECG) measures rhythm; an echocardiogram or other left ventricular ejection fraction (heart pumping measurement) (LVEF) test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

My overall treatment

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.



Days 1–21
Cobimetinib once daily plus vemurafenib twice daily.

Days 22–28
Vemurafenib continues twice daily; cobimetinib has its seven-day break.

A 28-day reference calendar

Day 1–21	Day 22–28
At home each day	Planned cobimetinib break

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety** NCCP / current product information / individual plan	BRAF, bloods including creatine phosphokinase (CPK)/creatinine, ECG, LVEF, skin and vision review.
During	**Safety checks** NCCP / current product information / individual plan	Bloods/CPK/creatinine monthly; LVEF month 1 then at least three-monthly; ECG four-weekly to week 12 then 12-weekly; monthly skin checks.
After / response	**Follow-up** NCCP / current product information / individual plan	After reduced-dose cobimetinib restart: LVEF at 2, 4, 10 and 16 weeks; skin/other-cancer surveillance and response scans separately planned.

Between treatments

- Report vision changes, breathlessness, bleeding or muscle pain with dark urine. Missed-dose windows: cobimetinib 12 hours; vemurafenib 4 hours; no extra after vomiting.

Bring medicine list, test dates and actual oncology/out-of-hours numbers. Example numbers must be replaced.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes / retina

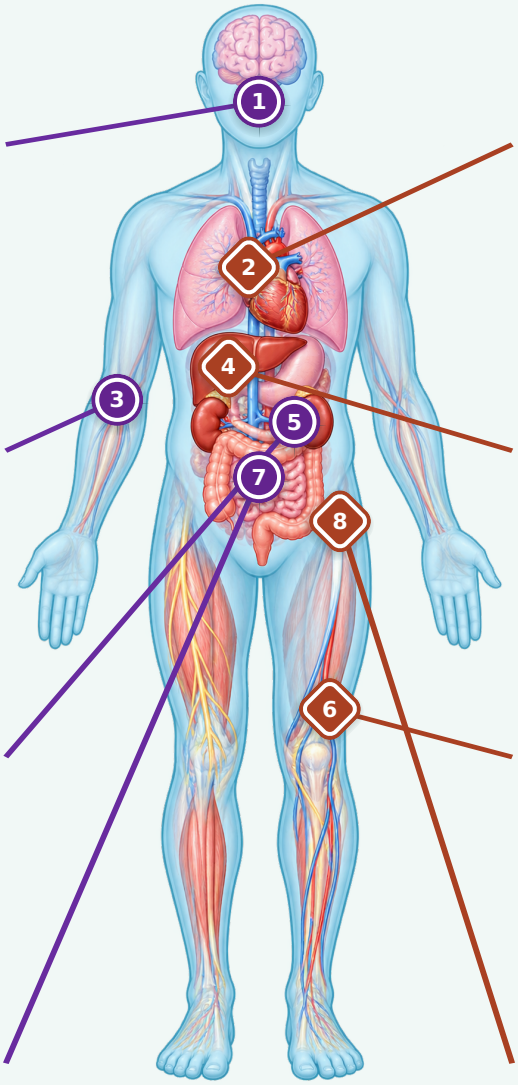
Cobimetinib + vemurafenib

New blurred vision, distortion, blind spots or eye pain can reflect retinal or inflammatory problems. Contact the team promptly rather than waiting for the next cycle.

2 Heart function / rhythm

Cobimetinib + vemurafenib

Cobimetinib can reduce heart pumping; vemurafenib can alter rhythm. Breathlessness, swollen ankles, palpitations or fainting need review.



3 Skin / sun

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Rash, photosensitivity and new skin lesions need protection and review. Severe blistering or peeling needs urgent help.

4 Liver

Cobimetinib + vemurafenib

Blood tests can detect liver injury before symptoms. Report jaundice, dark urine or upper-abdominal pain.

5 Kidneys / fluids

Cobimetinib + vemurafenib

Diarrhoea, vomiting or muscle damage can affect kidneys. Reduced urine or dark urine with muscle pain is urgent.

6 Muscles

Cobimetinib

Severe muscle pain, weakness or dark urine may signal muscle breakdown. The team may check muscle enzymes and kidneys.

7 Bowels

Cobimetinib + vemurafenib

Diarrhoea, nausea and abdominal discomfort can cause dehydration. Report persistent symptoms; severe pain or blood needs urgent assessment.

8 Bleeding

Cobimetinib

Unusual bruising, persistent nosebleeds, black or bloody stools, coughing blood or a severe new headache need prompt assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Cobimetinib + vemurafenib

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Phase: _____ Review plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My review record

Use the card as a treatment and urgent-contact record. There is no preset completion stamp count for this ongoing treatment.

Review	Date / initials

My treatment information

Latest review date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each completed review. Daily tablets continue according to your prescribed plan; a review stamp does not mean the course is complete or authorise further treatment. Record interruptions or a changed plan. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.